DELIRIUM



Postoperative delirium is one of the most common complications following major surgery. While many cases may be preventable, it may affect up to half of older adults and often goes unrecognized. It is associated with increased postoperative complications, length of stay in hospital, non-home discharge, mortality, and healthcare costs, as well as decline in function and cognition. [1-4]

Screening Tools

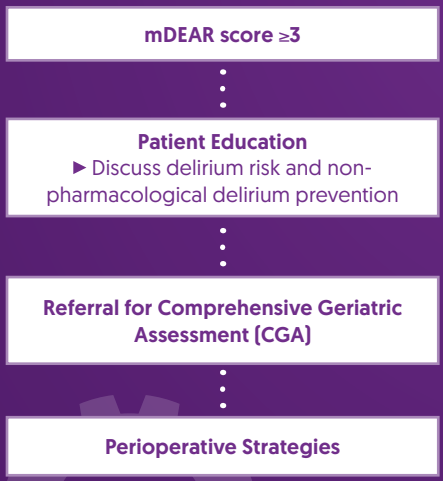
The Delirium Elderly At-Risk (DEAR) instrument has been used to predict postoperative delirium in elective and emergency orthopedic patients based on cognitive impairment, age, functional dependence, sensory impairment, and chronic substance use. “Among arthroplasty patients, having two or more risk factors was associated with an eight-fold increase in the incidence of delirium.” [1] The modified DEAR (mDEAR) uses routinely collected medical record data to assess cognitive impairment instead of the MMSE utilized in the DEAR and attributes 2 points to cognitive impairment and 1 point to each other factor. A patient scoring 3 or more indicates a higher risk of developing delirium. [5]

Modified Delirium Elderly at Risk (mDEAR) Tool

	YES	NO
1. Cognitive Impairment - Has the patient been disoriented to person, place, time or situation? - Does patient have a history of confusion or forgetfulness? - Does the patient have a history of cognitive impairment, memory loss, inattention or difficulty concentrating?	☐	☐
2. Patient Age - Age ≥ 80 years old	☐	☐
3. Sensory Impairment - Patient is hearing impaired; **OR** - Patient has low vision	☐	☐
4. Functional Status - Dependence in 1 or more activity of daily life (ADL)	☐	☐
5. Substance Use - Patient consumes > 3 drinks of alcohol per week; **OR** - Patient takes benzodiazepines > 3 times per week Examples include: Alprazolam (Xanax), Clonazepam (Klonopin), Diazepam (Valium), Lorazepam (Ativan) and Temazepam (Restoril)	☐	☐

SCORING
Assign 2 points for question 1, and 1 point each for questions 2-5

DELIRIUM ALGORITHM



Prehabilitation and Optimization Recommendations

Patient Education	⚙ Discuss perioperative risk of delirium ⚙ Non-pharmacological delirium prevention: • Reorientation • Access to natural light and a visible clock • Bring hearing aids and glasses and put them on in the daytime • Early mobilization • Family presence ⚙ Refer to online patient resources
Referral for Comprehensive Geriatric Assessment (CGA)	⚙ Several studies have shown that CGA-based care can reduce the risk of postoperative delirium. This is attributed to better identification of delirium risk factors and proactive initiation of multimodal delirium risk management in higher risk patients [6] ⚙ Refer to online provider resources for provincial geriatrician resources and telehealth consultation options
Perioperative Strategies	⚙ Minimize polypharmacy ⚙ Avoid prolonged fasting of fluids